



PROCEDURE

Pressure Injury Prevention and Management

Scope (Staff): All NMHS Mental Health (MH) Staff

Scope (Area): All NMHS MH Clinical Areas

Contents

1. Aim.....	2
2. Risk.....	2
3. Definitions	2
4. Key Points.....	3
5. Equipment.....	4
6. Community Mental Health Services	4
6.1 Assessing, Managing and Reporting Pressure Injury Risk	4
7. Inpatient Mental Health Services	4
Staff will consider the following risk factors when assessing pressure injury risk;	4
7.1 Pressure injury screening and assessment	5
7.2 Risk screening and assessment tools	6
8. Prevention of Pressure Injuries	6
8.1 Prevention Strategies	6
8.2 Strategies to assess mobility and complications associated with loss of mobility	7
9. Management of Pressure Injuries	7
9.1 Ongoing assessment.....	7
9.2 Management plan.....	8
9.2.1 Skin hygiene	8
9.2.2 Continence management	8
9.2.3 Nutritional status	9
9.2.4 Pain management.....	9
9.2.5 Repositioning and early mobilisation.....	9
9.2.6 Heel areas.....	9
9.2.7 Plantar and forefoot	10
9.3 Management of Pressure Injuries on Discharge.....	10
10. Communication	10
11. Discharge Planning	11
12. Documentation	11

13. Reporting Requirements	12
14. Compliance, monitoring and evaluation	13
Appendix 1: Guidelines for use of Support Surfaces.....	17
Appendix 2: Staging of Pressure Injuries	18
Appendix 3: Pressure Injury Alert Sticker	19
Appendix 4: Pressure Injury Risk Magnet used on Journey Board.....	19
Appendix 5: Flowchart on activating a consignment mattress at OPH.....	20
Appendix 6: Positions to reduce pressure.....	21
Appendix 7: Pressure Injury Prevention-PIP Tips for Prone Positioning	22
Appendix 8: Abbey Pain Assessment Scale (Follow on Assessment)	24
Abbey Pain Assessment Scale (Follow on Assessment) page 2	25

1. Aim

The aim of this procedure is to establish the minimum standards for screening, assessment prevention and management of pressure injury within inpatient mental health services.

North Metropolitan Health Service Mental Health (NMHS MH) is committed to preventing the development of pressure injuries in patients and effectively managing pressure injuries when they do occur, as required by the [National Safety and Quality Health Service Standards](#) and the [WA Pressure Injury Prevention and Management Guideline, 2017](#).

2. Risk

To reduce the risk of developing a pressure injury, all patients must be assessed for pressure injury risk and where indicated, pressure injury prevention strategies are to be implemented.

Non-compliance with this procedure will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☒
Medical Equipment	☐	Other	☐

3. Definitions

Pressure Injury (PI)	Localised damage to the skin and/or underlying tissue as a result of pressure or pressure in combination with shear. Pressure Injuries usually occur over a bony prominence but may also be related to a medical device or other object.
Friction	A mechanical force that occurs when two surfaces move across one another. Friction creates resistance between the skin and contact surface the person is lying or sitting on.

Braden Risk Assessment Tool	Validated pressure injury screening and risk assessment tool used in health care settings
DATIX Cims	Clinical Incident Management system used to notify clinical incidents at MH services.
iCM -iSOFT	Electronic Inpatient Clinical Management System
ISOBAR	Clinical handover framework used in NMHS Identify S ituation O bservations, B ackground, A greed plan, R eadback
KPI	Key Performance Indicator
HACs	Hospital Acquired Complications reported by Health Department as a mandatory service KPI
SSCD	State-wide Standardised Clinical Documentation

4. Key Points

- At risk community patients must be assessed for risk of pressure injury development using the Braden Scale. Risk must be documented and reported.
- Inpatients must be assessed for pressure injury risk within eight hours of admission using the Braden Scale and the Pressure Injury Risk Management Plan (PMR49) at minimum.
- Inpatients assessed as at risk for the development of pressure injuries must have the risk documented in the patient's clinical record, the PSOLIS Management Plan, as a PSOLIS risk alert and on the Patient Alert Form (PMR001).
- All inpatients identified at risk must have evidence-based prevention strategies implemented and documented as soon as possible, but within a maximum of eight hours of presentation.
- All inpatients identified at risk must have a daily skin assessment using the Comprehensive Skin Assessment tool (PMR28A) and an updated Pressure Injury Risk Management Plan (PMR49).
- Management of pressure injuries must focus on specific strategies to optimise healing and prevent deterioration or reoccurrence of the injury.
- Documentation requirements for patients with an identified pressure injury includes a Comprehensive Skin Assessment tool (PMR28A), a Pressure Injury Risk Management Plan (PMR49), a Pressure Injury Alert Sticker (**Appendix 3**) and the Wound Assessment and Care Plan (PMR28)
- All pressure injuries must be reported to relevant senior nursing staff and via the Datix Clinical Incident Management System. This applies to both inpatient and community settings.

5. Equipment

- Standard Dressing Trolley
- Sterile dressing packs
- Prescribed cleaning solution

6. Community Mental Health Services

Healthcare professionals from community services must ensure that they know how to assess pressure injury risk, using the Braden Scale, if clinically indicated.

6.1 Assessing, Managing and Reporting Pressure Injury Risk

For community patients living independently, who are assessed either as at risk for developing a pressure injury, or already having a pressure injury, the community healthcare professional must:

- Provide education to the patient and/or carer on regular skin inspection, prevention strategies and/or pressure injury management
- Liaise with the patient's General Practitioner (GP) and other care providers for ongoing prevention strategies and pressure injury management
- Document in the clinical record any consultation and/or referral to GP, or other health care providers, in regard to ongoing pressure injury management
- Document the risk or existing pressure injury as per Section 10 'Communication' and Section 12 'Documentation' of this Policy
- Report any pressure injuries as per Section 13 of this Policy: 'Reporting Requirements'

7. Inpatient Mental Health Services

Staff will consider the following risk factors when assessing pressure injury risk;

- Skin status (e.g. quality of skin, dry, mottled, presence stage I likely to develop to stage II PI)
- Body temperature (elevated)
- Perfusion, circulation and oxygenation factors
- Nutrition status
- Sensory perception limitations
- Activity and mobility limitations
- Moisture
- Older age
- Blood markers (e.g. low serum albumin levels, low haemoglobin, elevated C-reactive protein)
- General and mental health status

All patients must be screened and assessed for pressure injury **as soon as possible** and within **8 hours of admission** by medical and/ or nursing staff using both clinical judgement and the [Braden Scale for Predicting Pressure Sore Risk](#).

Medical staff assess the immunisation status of the patient.

All staff adhere to infection control principles including Aseptic Technique ([Refer to NMHS Infection Prevention and Control Policy](#)).

A comprehensive skin assessment must be conducted with attention to:

- Skin over bony prominences including heels, sacrum, ischial tuberosities, femoral trochanters, elbows, malleoli, spinous processes, occiput, and ears;
- Around and under medical and therapeutic devices (such as Continuous Positive Airway Pressure masks, endotracheal tubes, oxygen tubing, splints or casts, and neck immobilisation collars);
- Presence of persistent erythema or blisters;
- Localised pain especially at pressure points;
- Signs of maceration, paying attention to skin folds especially for patients with obesity;
- Localised heat, localised oedema and induration; and
- Purplish / bluish discolouration in those patients with darkly pigmented skin.

Patient with impaired activity, mobility or sensory perception must be considered at risk for a pressure injury.

Patients identified “at risk” of, or with an existing pressure injury will be considered for referral to a wound management specialist.

Interventions are initiated and identified in the Braden Pressure Risk Injury Management Plan (PMR49) and documented in the management plan and in the medical record.

Management plans are updated daily to provide accurate and current information to facilitate the handover of care (Refer to [DoH Clinical Handover Policy](#)) and clinical alerts are updated as required (**Appendix 3** and **Appendix 4**).

Medications are managed in accordance with the requirements of the [NMHS Administration of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Policy](#) and the [MH Medication Prescribing, Administration and Management for inpatients](#).

7.1 Pressure injury screening and assessment

Risk screening and assessment must be completed for **ALL** inpatients within eight hours of admission. If this is not possible, screening and assessment must be completed and documented as soon as is practicable and reasons for the delay documented in the clinical record.

- Risk screening and assessment must be completed for all patients 65 years and over, every week for 4 consecutive weeks following admission, provided that they have been assessed as being low risk for pressure injuries.
- Daily skin assessments must be undertaken for patients identified as at risk of pressure injury.
- Skin assessments must be undertaken for patients in the following circumstances:
 - When a patient has prolonged periods of sitting or bed rest
 - Following a clinical change or deterioration in mental state, medical/physical or cognitive condition

-
- Following a change in the patient's mobility
 - Annually for long stay patients, unless otherwise clinically indicated
 - When a patient identified as being at risk is transferred or discharged

7.2 Risk screening and assessment tools

The purpose of a screening tool is to identify that there is an issue and then refer on appropriately.

A comprehensive skin risk assessment is undertaken using the Pressure Injury Risk Management Plan (PMR49).

The PMR49 is used as a screening tool, which has a section on nutrition with a process for referring to the Dietetics Service if the **Braden Score < 3**.

Dietetics may use the Mini Nutritional Assessment – Short Form (MNASF) that is validated for use in elderly patients.

8. Prevention of Pressure Injuries

All patients identified at risk of pressure injuries must have evidence-based prevention strategies implemented and documented as soon as possible, but within a maximum of eight hours of presentation.

The Clinical Nurse Specialist (CNS) must be notified if the risk assessment score indicates significant risk, or the risk level increases at any time. Braden Score **6-18** indicates risk.

Patients assessed as at risk must have the risk documented in the:

- Patient's clinical record. This must include the Braden Scale score.
- PSOLIS Management Plan. This must include all individualised prevention and management interventions.
- PSOLIS Risk Alert, under the category of 'Medical Alert'.
- Patient Alert Form (PMR 001) as a 'Clinical Alert'.

8.1 Prevention Strategies

Prevention strategies include (but are not limited to):

- Implementation of an individualised Pressure Injury Risk Management Plan (PMR49) which is tailored to the individual, addresses their specific risk factors and is focussed on prevention and optimising healing.
- The care plan should be developed in collaboration with the patient and/or carer and their involvement and preferences documented in the medical records. If collaboration is not possible the reason should be recorded in the patient's clinical record.

- Daily comprehensive skin assessment using the Comprehensive Skin Assessment tool (PMR28A) with any skin changes documented. The PMR28A must be completed daily if the patient has been assessed as at risk of developing, or already has, a pressure injury.
- Use of soap-free pH neutral (pH4-6) skin cleansers, treatment of dry skin, avoidance of excess contact with bodily fluids, and a continence program where necessary.
- Regular assessment and monitoring of fluid and nutritional intake to ensure adequate hydration and nutrition (Fluid Balance Chart – Input & Output)
- Referral to the Dietician if dietary intake is inadequate as indicated by a Braden Nutrition score less than or equal to 3 (PMR49). The Dietitian performs a full nutritional assessment and identifies dietary requirements.

8.2 Strategies to assess mobility and complications associated with loss of mobility

- Scheduled repositioning regime and use of a repositioning chart where necessary.
- Provision of pressure relieving devices as clinically indicated.
- Correct manual handling techniques by staff to avoid friction and shear.
- Mobility / activity promotion consistent with the patient's medical condition, and physical / cognitive ability.
- Referral to other health professionals as clinically indicated e.g. Physiotherapy, Podiatry, Occupational therapist, Specialist Nurse Consultant (SCGOPH).
- Correct fitting, removal and checking of devices / orthoses / shoes / anti-embolic stockings and casts.
- Management of pain. Abbey Pain Assessment Scale for assessment of pain.
- Skin protection, moisture reduction and optimal skin hygiene and temperature.
- Adequate nutrition and hydration, including nutritional supplements (where indicated).
- Continence management.
- Ongoing patient and/or carer education regarding risk factors for developing pressure injuries, strategies for prevention and the importance of skin inspection and care, which is tailored to meet their specific health literacy needs.

9. Management of Pressure Injuries

Management of pressure injuries must focus on specific strategies to optimise healing and prevent deterioration or reoccurrence of the injury.

9.1 Ongoing assessment

Ongoing assessment and documentation of the pressure injury must include:

- Minimum of weekly wound review and monitoring, unless otherwise clinically indicated or as per specialist advice

- Classification of the stage of pressure injury according to the NPAUP-EPUAP Pressure Injury Staging Chart (**Appendix 2**)
- Anatomical location, measurements (size/depth), exudate, odour, condition of surrounding skin, level of pain, signs of clinical infection, tunnelling, sinus tracts and appearance of the wound bed
- Photographic record of the injury must occur on initial assessment; as the injury changes or as per specialist advice and filed in the patient's clinical record ([NMHS Clinical Photography Policy](#)).
- Reassessment should occur at each dressing change.

The NMHS MH Wound Assessment and Care Plan (PMR28) must be initiated for all pressure injuries and clinically relevant wound management strategies implemented as per the NMHS [MH Wound Management Procedure](#). The Wound Assessment and Care Plan should consider:

- Collaboration with other health professionals, such as a wound management specialist, (available through Sir Charles Gairdner Hospital at WoundManagement.SCGH@health.wa.gov.au), for assessment and contribution to care planning and management as clinically indicated
- Assessment, documentation and management of pain
- Nutritional support to optimise wound healing
- Review of wound management should healing not proceed in an expected or timely manner.
- Collaboration with other health professionals, such as wound management specialists, allied health, infection prevention specialists, pain specialists, discharge coordinators, for their assessment and contribution to care planning and management should occur as clinically indicated.

9.2 Management plan

Nursing will consider the following when developing management plans in collaboration with the MDT.

9.2.1 Skin hygiene

- Keep the skin clean and appropriately hydrated
- Cleanse the skin promptly after incontinence
- Avoid the use of alkaline soaps and cleansers
- Protect the skin from moisture, consider use of barrier products
- Check all skin folds including underneath the pannus in obese patients
- Avoid vigorous rubbing of skin

9.2.2 Continence management

- Use high absorbency incontinence products to protect skin if patient incontinent
- Review continence products/aids regularly
- Implement care hygiene program for at risk patients (e.g. UTIs and further deterioration)
- Refer to continence service as required
- Inspect skin daily underneath dressing. If the dressing becomes soiled (e.g. due to

incontinence) it must be removed and replaced.

9.2.3 Nutritional status

- Assess, encourage/assist and monitor nutrition and hydration status to optimise healing and tissue repair.
- Malnutrition Screening Tool (MST) is used to screen for malnutrition.
- Refer patients identified as at risk of malnutrition to a Dietitian for comprehensive assessment.
- Follow individualised nutrition action plans, monitor and document outcomes.

9.2.4 Pain management

- Pain is assessed at least every shift/episode of care/clinic review using a validated tool (Abbey Pain Assessment Scale, **Appendix 8**)
- A pain management plan is developed with the treating team (where possible includes collaboration with the patient, family and carers) that identifies analgesics prescribed, timing of analgesics and care requirements with dressing changes, manual handling and repositioning.

9.2.5 Repositioning and early mobilisation

- Determine repositioning frequency with consideration to the patients:
 - Skin and tissue tolerance
 - General medical condition
 - Overall treatment objectives
 - Comfort and pain

Reposition the individual in such a way that optimal offloading of all bony prominences and maximum redistribution of pressure is achieved (Refer to Guidelines for Use of Support Surfaces (**Appendix 1**))

- Use the 30° sidelying position in preference to 90° (See **Appendix 6**).
- Avoid positioning patients on areas of erythema, if practicable.
- Reposition the individual using manual handling techniques and equipment that reduce friction and shear e.g. slide sheets, slide board, turn assist, HoverMatt.
- Promote sitting out of bed in an appropriate chair with a specialist cushion for limited periods of time. Ensure that the patient is positioned correctly e.g. postural alignment and support, even weight distribution, balance and stability using functions such as tilt in space, back angle recline, adjustable seat dimensions, lateral supports, leg rests (See **Appendix 5** Flow Chart on Activating a consignment mattress at OPH).
- If unable to sit out of bed, utilise pressure injury aids and monitor and record repositioning.
- Consider patients bed positioning regime: supine/ prone/ axillar (e.g. in prone check toes, dorsum of foot, genitalia, sternum, chin, forehead and cheekbones) (See **Appendix 9** Prone Positioning).
- Implement an early mobilisation program that increases activity and mobility as soon as clinically indicated.

9.2.6 Heel areas

- Use heel off-loading devices e.g. heel elevators/wedges/pillows when lower leg /



foot assessment indicates that a patient is at risk of developing a pressure injury or has a pressure injury.

- Offload the heel completely in such a way as to distribute the weight of the leg along the calf without placing pressure on the Achilles tendon or popliteal vein.
- Use available equipment/technique when moving the patient up the bed to minimise injury or pain or discomfort.

9.2.7 Plantar and forefoot

- Use service endorsed equipment and manual handling techniques when moving the patient up the bed to minimise injury or pain or discomfort.
- Check that the patient's foot is not pressed up against the end of the bed
- Consider adding in a bed extension on the bed if the patient is too tall for the bed.

9.3 Management of Pressure Injuries on Discharge

If a patient is discharged from the service whilst they have an unhealed pressure injury, the discharge summary should include:

- The current Braden Score.
- Current details about the pressure injury and associated wound management plan.
- Relevant and ongoing pressure injury prevention strategies.
- Referral to appropriate services for ongoing management.

10. Communication

Clinical handover must be in accordance with the [DoH Clinical Handover Policy](#) MP0095.

With the permission of the patient /Guardian identified risks and management plans are shared with family, carers and others involved in their ongoing care.

Communication of the level of risk, the care plan and effectiveness of the care must be part of all handover occasions, including discharge and transfer.

Identified Risk and agreed management plans are highlighted on the Journey Board and in Clinical Handover (using iCM/iSoft).and in accordance with the [DoH Clinical Handover Policy](#).

This information must be available to all relevant members of the multidisciplinary team, as well as patients and/or carers.

Management plans should be developed collaboratively with patients and carers:

- Staff will provide education to patients, families and / or carer on the causes, prevention, and treatment of pressure injuries.
- Patients, families and carers will receive information about [Aishwarya's CARE Call](#) 3-step clinical escalation process.

Key messages for patients, families and carers

Patients, families and/ or carers should be considered as active participants in the prevention and management plan. They should receive information about the relevant risk factors and how to prevent pressure injuries. Patients need to be encouraged to self-reposition and maintain mobility and activity where possible

11. Discharge Planning

Where patients require a continuation of management of their pressure injury referral to the appropriate clinical services on discharge the following should be performed.

1. Ensure referral and current pressure injury management plan are supplied to the service provider.
2. Complete a written discharge management plan including the pressure injury management plan, the Braden score on discharge and any risks identified. The discharge management plan is given to the patient and /or family and carer with the permission of the patient/guardian. Document in the medical record that this information was provided and if it was not provided documents the reason/s. Refer to Triage to Discharge Care Transfer Summary (SMHMR916) and the Treatment Support and Discharge Plan (SMHMR907) ([SSCD Triage to Discharge](#)).
3. Provide written and verbal education to the patient, family and/or carer regarding adverse signs and symptoms that a need for further review:
 - Infection
 - Pressure area deterioration
 - Other potential problems, e.g. pain or bleeding
4. Provide additional dressing supplies to maintain pressure injury care until their next review
5. Ensure the patient/carer are aware of any follow-up appointments and/or service referrals.
6. Provide information and relevant education material to the patient, family, carer and others involved in their care.

12. Documentation

- Documentation related to assessment, prevention strategies and wound management strategies must be maintained and contemporaneously updated. Documentation must include:
 - Braden Scale
 - Individualised Pressure Injury Risk Management Plan (PMR49)
 - Comprehensive Skin Assessment (PMR28A)
 - Updated clinical records, inclusive of PSOLIS Management Plan
 - Clinical risk alerts
 - PSOLIS Management Plan

-
- Where required, such as in the identification of a pressure injury, additional documentation must include:
 - Individualised Wound Assessment and Care Plan (PMR28)
 - Pressure Injury Alert Sticker (**Appendix 3**) as per the WA Health Pressure Injury Alert Sticker: Guidelines for Use
 - Patients and/or carers must be informed if a pressure injury occurs during admission and a collaborative individualised patient care plan developed.
 - When clinically indicated, information and resources on pressure injury prevention and management strategies must be available to all patients and / or carers on admission to the service and upon discharge. Consumer / carer endorsed resources can be found at:
 - WA Department of Health Patient First
 - NMHS MH Safety, Quality and Performance Unit
 - Health Consumer Council
 - Staff education and resources can be found at:
 - WA Health Pressure Injury Prevention
 - NMHS Mental Health Take 5 Library

13. Reporting Requirements

All pressure injuries must:

- Be initially reported to the CNS of the ward (for inpatient services) or the Team Leader (for community services)
- Have a Pressure Injury Alert Sticker (**Appendix 3**) placed in the patient's clinical record. The sticker should contain the following minimum information:
 - Stage and location of the pressure injury
 - Whether the pressure injury was present on admission
- Be reported by submitting a clinical incident report via the Datix Clinical Incident Management System (CIMS) and investigated as per the NMHS MH Clinical Incident Management Procedure, for all:
 - Hospital acquired pressure injuries
 - Any pressure injury that has deteriorated to the next stage of injury (including those present on admission)
 - **Note:** Pressure injuries identified on admission do NOT need to be reported via Datix CIMS, unless they deteriorate to the next stage of injury
 - If relevant, pressure injuries already present on admission (following transfer), must be investigated by the organisation where the pressure injury occurred
- The NMHS MH Safety, Quality and Performance Unit are responsible for collating, reporting and monitoring pressure injury incidence data, to assess compliance against the performance indicators (below) and to facilitate ongoing quality improvement.

14. Compliance, monitoring and evaluation

- 0% hospital acquired pressure injury incidents.
- 0% of pressure injuries present on admission deteriorated to the next stage of injury.
- 100% of inpatients will have evidence of Pressure Injury Risk Assessment within eight hours of admission.
- 100% of patients, identified as at risk for the development of a pressure injury, will have a daily comprehensive skin assessment completed and documented.
- 100% of inpatients with a pressure injury, or assessed as at risk, will have an individualised Patient Care Plan with clinically relevant interventions.
- 100% of patients with a pressure injury, or assessed as at risk will have a clinical risk alert entered on PSOLIS.
- 100% of patients with a pressure injury will have an individualised wound management plan with clinically relevant interventions.
- 100% of patients with a pressure injury will have an alert sticker places in the patient's clinical record.
- Leadership Committees receive service data reports.
- The MHPHDS Safety, Quality and Performance report on audit results to the Mental Health Management Committee and MHPHDS Executive (Link to [SQP Audit tools](#)).

Related internal policies, procedures and guidelines

[NMHS MH Management of Clinical Alerts](#)

[NMHS MH AW Wound Care Infection Prevention and Control Guidelines](#)

[SCGOPHCG Infection Prevention and Control Manual - Aseptic Technique Policy](#)

[SCGOPHCG Wound Management Nursing Practice Guideline -NPG16](#)

[SCGH Clinical Photography Policy No.180](#)

[MHPHDS Aseptic Technique Policy](#)

[MHPHDS Hand Hygiene Policy](#)

[NMHS Clinical Handover Policy](#)

[NMHS eReferral Policy](#)

[MH Identification of Patient Procedure](#)

[NMHS Infection Prevention and Control Policy](#)

[NMHS Administration of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Policy](#)

[MH Medication Prescribing, Administration and Management for Inpatients](#)

[Aishwarya's CARE Call \(NMHS Hub\)](#)

[DoH SSCD Triage to Discharge](#)

References

[Australian Commission on Safety and Quality in Health Care](#)

[Braden Scale for Predicting Pressure Sore Risk](#)

[WA Health Clinical Handover Policy](#)

[WA Pressure Injury Prevention and Management Clinical Guideline](#)

[MHPHDS Mandatory Training Policy](#)

[ACSQHC Fact Sheet Preventing Pressure Injuries and Wound Management](#)

[Infection Prevention and Control in Western Australian Healthcare Facilities](#)

[Australian Guidelines for the Prevention and Control of Infection in Healthcare \(2019\)](#)

Useful resources

[Prevention and Treatment of Pressure Ulcers/Injuries Clinical Practice Guideline. The International Guide](#)

[Patient First](#)

[Wounds Australia](#)

[European Pressure Ulcer Advisory Panel, National Pressure Ulcer Advisory Panel and Pan Pacific Pressure Injury Alliance. Prevention of Treatment of Pressure Ulcers/injuries: Clinical Practice Guideline. The International Guideline. Emily Haesler \(3rd ED.\). EPUAP/NPIAP/PPPIA: 2019](#)

Policy Sponsor	Director of Nursing MHPHDS				
Policy Contact	Director of Nursing MHPHDS				
Date First Issued:	22/12/2012	Last Reviewed:	14/07/2022	Review Date:	14/07/2025
Approved by:	Director of Nursing MHPHDS			Date:	16/06/2022
Endorsed by:	Director of Clinical Services MHPHDS			Date:	14/07/2022
NSQHS Standards Applicable:	☐  Std 1: Clinical Governance ☐  Std 2: Partnering with Consumers ☒  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety ☒  Std 5: Comprehensive Care ☐  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☒  Std 8: Recognising and Responding to Acute Deterioration				
National Standards for Mental Health Services	☐ Std 1: Rights and Responsibilities ☐ Std 2: Safety ☒ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☒ Std 5: Promotion and Prevention ☐ Std 6: Consumers ☐ Std 7: Carers ☐ Std 8: Governance, leadership and management ☐ Std 9: Integration ☒ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry				
Chief Psychiatrist's Standards for Clinical Care:	Care Planning, Physical Health Care of Mental Health Consumers, Risk Assessment and Management, Transfer of Care				
Printed or personally saved electronic copies of this document are considered uncontrolled					

Document Version Control			
Date	Version	Author	Notes
22/12/2012	1.0	MHEG	Meeting Minutes
21/09/2018	2.0	Senior Research Nurse, MH	Policy title change to Pressure Injury Prevention and Management. Template, references, and hyperlinks updated, inclusion of community services in the policy scope, at risk community patients to be assessment for pressure injury risk using Braden scale, inpatients to be assessment for pressure injury using Braden scale within 8 hours of admission. All inpatients identified at risk must have a daily skin assessment, update of documentation requirements for patients with an identified pressure injury. New flow chart added.
13/07/2022	3.0	Director of Nursing MHPHDS	Scheduled review, information provided to Nursing Services with a single set of procedures to guide and inform Clinicians in Pressure Injury Prevention and Management, template and links updated.
14/04/2023	3.1	Director of Nursing MHPHDS	Update of Braden Pressure Ulcer Risk Assessment Tool



The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative (**ISD 800**).

This document can be made available in alternative formats on request for a person with a disability.

© North Metropolitan Health Service 2021

Copyright to this material is vested in the State of Western Australia unless otherwise indicated. Apart from any fair dealing for the purposes of private study, research, criticism or review, as permitted under the provisions of the *Copyright Act 1968*, no part may be reproduced or re-used for any purposes whatsoever without written permission of the State of Western Australia.



Appendix 1: Guidelines for use of Support Surfaces

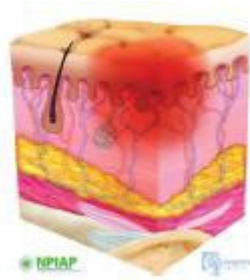
NOT At Risk	Bed Device	Chair Device
No pressure injury present	- Standard hospital foam mattress. Range 247kg-254kg - Bariatric foam (0-450kg) - Tempura-Med (memory foam) Range 20kg-150kg - Compella (113-454kg)	
At Risk to Moderate risk	Bed Device	Chair Device
Pressure injury stage 1-2	- Tempur-Med (memory foam) Range 20kg-150kg <u>Alternating pressure mattresses</u> - Talley Quattro Acute (0-250kg) - Talley Quattro Plus (0-200kg) - Talley Quattro Overlay (0-200kg) - Alpha Response Overlay (40-135kg) - Alpha Response Mattress (40-160kg) or <u>Constant low pressure/air loss mattresses</u> - VersaCare (30-227kg) - Progressa (113-455kg) - Compella (113-454kg)	- Foam cushions - Basic – 100kg - ApolloHealthcare - 152kg - Dyna-Tek Owl - 152kg - Roho cushion Size S, M, L, XL - Gel Tru-Life – 130kg - Refer to Wound Management or OT for custom made cushion requirements
High to Very High Risk	Bed Device	Chair Device
Pressure injury stage 2 and above; and/or multiple pressure injuries. Inability to keep patient positioned off pressure injury	<u>Alternating pressure mattresses</u> - Quattro Acute (0-250kg) or <u>Constant low pressure/air loss mattresses</u> - VersaCare (30-227kg) - Talley Quattro Acute (0-250kg) - Talley Quattro Plus (0-200kg) - Compella (113-454kg) - Progressa (113-455kg)	- Foam cushions - Basic – 100kg - ApolloHealthcare - 152kg - Dyna-Tek Owl - 152kg - Roho cushion Size S, M, L, XL - Gel Tru-Life – 130kg - Refer to Wound Management or OT for custom made cushion requirements
A complete patient assessment (i.e. weight, body shape, Waterlow Risk Assessment Tool, pressure injury staging, mobility) is required before selecting the bariatric integrated low air loss /constant low-pressure mattress (Compella) with turning function. Bariatric foam (450kg) is available from the Hospital Equipment Service (HES) if the Compella bed is not required. Patients with spinal injury or multiple fractures - DO NOT use pressure redistribution support surfaces without medical approval. Contact the Wound Management Service, HES or After-Hours CNS if further advice is required eg low weight/cachectic or bariatric patients.		



Appendix 2: Staging of Pressure Injuries

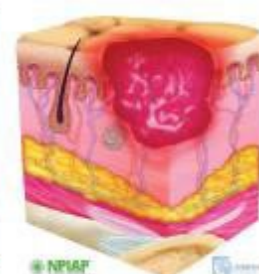
STAGE 1:

Intact skin with non-blanchable redness of a localized area usually over a bony prominence. The area may be painful, firm, soft, warmer or cooler compared to adjacent tissue.



STAGE 2:

Partial thickness skin loss involving the dermis. The pressure injury may also present as an intact or open/ruptured serum-filled blister. Presents as a shiny or dry, shallow ulcer without slough or bruising.



STAGE 3:

Full thickness skin loss involving damage or necrosis of subcutaneous tissue that may extend down to but not through, underlying fascia. Slough may be present but does not obscure the depth of tissue loss.



STAGE 4:

Full thickness skin loss with extensive destruction, tissue necrosis or damage to muscle, bone or supporting structures (for example, tendon or joint capsule).



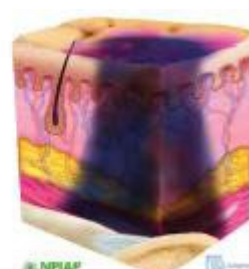
UNSTAGEABLE PRESSURE INJURY: DEPTH UNKNOWN

Full thickness tissue loss in which the base is covered by slough and/or eschar. Until the slough and eschar is removed the true depth and stage cannot be determined.



SUSPECTED DEEP TISSUE INJURY: DEPTH UNKNOWN

Purple or maroon localized area or discoloured, intact skin or blood-filled blister due to damage of underlying soft tissue.



Source: European Pressure Ulcer Advisory Panel, National Pressure Ulcer Advisory Panel and Pan Pacific Pressure Injury Alliance. Prevention of Treatment of Pressure Ulcers/injuries: Clinical Practice Guideline. The International Guideline. Emily Haesler (3rd ED.). EPUAP/NPIAP/PPPIA: 2019.

Appendix 3: Pressure Injury Alert Sticker

Pressure Injury Alert

Was the pressure injury present on admission? Yes No

Anatomical location of pressure injury: _____

Circle the pressure injury classification stage:

Suspected
I, II, III, IV, Unstageable, Deep Tissue

Has a wound management plan been commenced? Yes No

Has a pressure management plan been commenced? Yes No

Has a Datix CIMS been notified? Yes No

CIMS Ref Number: _____

Name, designation & signature Date: _____ Time: _____

NB. This sticker is NOT to be used for Mucosal Pressure Injuries



Please use
one sticker per
pressure injury

Appendix 4: Pressure Injury Risk Magnet used on Journey Board



Appendix 5: Flowchart on activating a consignment mattress at OPH

ACTIVATING A CONSIGNMENT (RENTAL) TALLEY QUATTRO MATTRESS

Nursing staff to assess the patient's risk of pressure injury and establish the need for Talley Quattro air mattress

If non available from OPH stock, request PSA/nursing staff to get mattress from Equipment Storage Room (E Block, Level 1). Provide them with patient sticker or patient name

PSA/ nursing staff collects the requested Talley Quattro Plus mattress, pump and red plastic bag from the storage area and completes the "Consignment Log Sheet" kept with the mattresses. It is important to place the patient sticker/write patient's name on the log sheet in the relevant space.



PSA/staff transports the mattress and the pump to the ward and set it up on the bed.

Mattress is to be placed directly onto a bed frame, **not on top of any existing mattress**. Allow 10-15 minutes for mattress to inflate before placing the patient on the mattress. The pump will display message "ACTIVE MEDIUM" when the setup process is complete

If there are any issues or concerns with the air mattress, return **the mattress and the pump** to the storage area with the note of what the issue is and take a new mattress. **Complete the "Consignment Log Sheet"** again for new mattress.

Area Manager to activate the rental with HospEquip on the next business day

When no longer required place the mattress in red plastic bag and return to the Equipment Storage Room. Fill in Return Date on "Consignment Log Sheet".

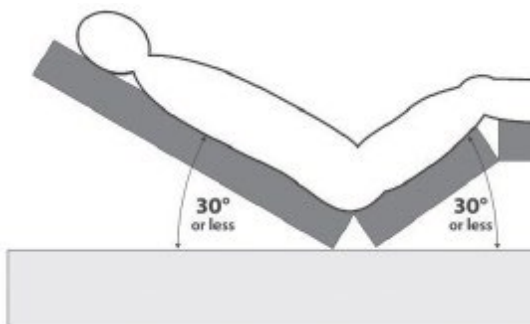


Appendix 6: Positions to reduce pressure

30 side lying position



Head of bed elevation

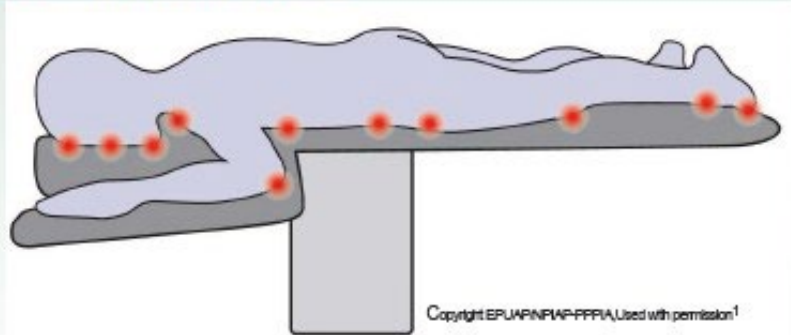


Appendix 7: Pressure Injury Prevention-PIP Tips for Prone Positioning

- General Tips:**
- Use a pressure redistribution surface (for those not on a bed specifically designed for proning)
 - Follow manufacturer instructions when using beds, positioning devices, prophylactic dressings and other products.
 - Positioning devices/pillows are needed to offload pressure points.
 - Involve enough trained staff to avoid friction-shear when repositioning. May reposition into swimmer position.
 - Microshifts and small position changes should be performed while prone, especially in non-rotating beds.
 - Assess all pressure points :
 - Prior to proning (anterior surfaces). Prior to returning to supine position (posterior surfaces).
 - When alternating arm position in swimming arm position, assess integrity of skin of arm/head/face.
 - Document all skin assessments and preventive measures.

Pressure Points:

- Forehead
- Chin
- Cheeks
- Nose
- Clavicle - shoulder
- Elbow
- Chest - breasts
- Genitalia - penis
- Anterior pelvic bones (iliac crests, ischium, symphysis pubis)
- Knees - patella
- Dorsal feet and toes¹
- Under/around medical devices



Pay special attention to the following areas:



HEAD

- Apply soft silicone multi-layered foam prophylactic dressings to pressure points on face.
- Manage moisture: Suction oral secretions. Use liquid skin protectants/sealants on face. Change foam dressing pm. Apply hydrofiber/calcium alginate dressing under prophylactic dressing to manage excess moisture.
- Apply thin foam dressings under medical devices. Avoid multiple layers of dressings that increase pressure.
- Offload head with offloading device(s): Consider the density of foam, height of the cushion, angle of the face, and endotracheal tube (ETT) positioning when selecting an appropriate device.
- With manual proning, shift patient's head q 2 hours; re-position head q 4 hours. May adjust timing to patient needs.
- Note: commercially available ETT securement devices may contribute to increased skin breakdown in prone patients. Assess skin carefully. Consider tape to secure ETT during proning.
- Maintain eye care to prevent corneal abrasions. Apply ophthalmic lubricant. Tape eyelids shut horizontally.
- Ensure tongue is inside patient's mouth. A small soft bite block may help. Assess tongue for injury.



TORSO

- Place EKG leads on back while proning.
- Apply prophylactic foam dressings to pressure points.
- Ensure central lines, arterial lines and cannulas are secured (e.g., sutured).
- Empty ileostomy/colostomy pouches and pad around stoma site.
- If receiving enteral feedings, turn off feeding 1 hour before prone position turn. Resume once in prone position as ordered.
- Secure all tubes and devices away from skin; protect surrounding skin with prophylactic dressings and bridge areas with positioning devices.
- Create channels for tubes with positioning aids. Ensure that there are no unsecured devices under the torso.



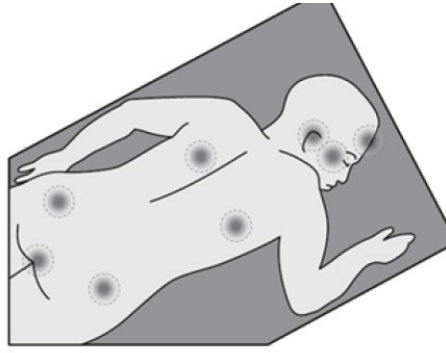
LEGS

- Apply prophylactic foam dressings to pressure points (e.g., patella and pretibial area).
- Remove securement devices and align urinary catheter/fecal management device toward foot of bed.
- Ensure that there are no unsecured devices under legs. Offload feet.

BREASTS & GENITALIA are particularly sensitive tissues that should be offloaded and protected.

Swimming/Freestyle Position

- Patients in standard prone position can be repositioned using the swimming position.
- Alternate the position of the arms and direction of the head, in a manner that is similar to that of a freestyle swimmer.
- Pressure points on the diagram correspond to the body surface facing the mattress.



Copyright EPUAP/NPIAP-PPPIA, Used with permission¹



Appendix 8: Abbey Pain Assessment Scale (Follow on Assessment)

ABBEY PAIN ASSESSMENT SCALE (FOLLOW ON ASSESSMENT)

DATE AND TIME	DATE AND TIME	DATE AND TIME	DATE AND TIME	DATE AND TIME	DATE AND TIME	DATE AND TIME	DATE AND TIME	DATE AND TIME	DATE AND TIME	DATE AND TIME
VOCALISATION eg: whimpering, groaning, crying <i>Absent 0 Mild 1 Moderate 2 Severe 3</i>										
FACIAL EXPRESSION eg: looking tense, frowning grimacing, looking frightened <i>Absent 0 Mild 1 Moderate 2 Severe 3</i>										
CHANGE IN BODY LANGUAGE eg: fidgeting, rocking, guarding part of body, withdrawn <i>Absent 0 Mild 1 Moderate 2 Severe 3</i>										
BEHAVIOURAL CHANGE eg: increased confusion, refusing to eat, alteration in usual patterns <i>Absent 0 Mild 1 Moderate 2 Severe 3</i>										
PHYSIOLOGICAL CHANGES eg: temperature, pulse or blood pressure outside normal limits, perspiring, flushing or pallor <i>Absent 0 Mild 1 Moderate 2 Severe 3</i>										
PHYSICAL CHANGES eg: skin tears, pressure areas, arthritis, contractures, previous injuries <i>Absent 0 Mild 1 Moderate 2 Severe 3</i>										
Total score =										
Signature of person completing score										
0-2 NO PAIN	3-7 MILD PAIN			8-13 MODERATE PAIN			14 + SEVERE			

The Pain Scale is an instrument designed to assist in the assessment of pain in patients who are unable to clearly articulate their needs.

The Abbey pain scale does not differentiate between distress and pain, therefore measuring the effectiveness of pain relieving interventions is essential.

The pain scale should be used as a movement based assessment, therefore observe the patient while they are being moved, during pressure area care, while showering etc.

A second evaluation should be conducted **1 hour after** any intervention taken. If, at this assessment, the score on the pain scale is the same, or worse, consider further intervention and act as appropriate.

Complete the scale hourly until the patient scores mild pain then 4 hourly for 24 hours treating pain if it recurs.

If the pain/distress persists, undertake a comprehensive assessment of all facets of the patients care and monitor closely over 24 hours including further intervention undertaken.

If there is no improvement during that time notify the doctor/pain team of the pain scores and actions taken.



Abbey Pain Assessment Scale (Follow on Assessment) page 2

Addressograph

Abbey Pain Scale For measurement of pain in patients who cannot verbalise.

Name and designation of person completing the scale:

Date: Time:

How to use scale: While observing the patient, score questions 1 to 6

Q1. Vocalisation

eg. whimpering, groaning, crying

Absent 0 Mild 1 Moderate 2 Severe 3

Q1

☐

Q2. Facial expression

eg: looking tense, frowning, grimacing, looking frightened

Absent 0 Mild 1 Moderate 2 Severe 3

Q2

☐

Q3. Change in body language

eg: fidgeting, rocking, guarding part of body, withdrawn

Absent 0 Mild 1 Moderate 2 Severe 3

Q3

☐

Q4. Behavioural Change

eg: increased confusion, refusing to eat, alteration in usual patterns

Absent 0 Mild 1 Moderate 2 Severe 3

Q4

☐

Q5. Physiological change

eg: temperature, pulse or blood pressure outside normal limits, perspiring, flushing or pallor

Absent 0 Mild 1 Moderate 2 Severe 3

Q5

☐

Q6. Physical changes

eg: skin tears, pressure areas, arthritis, contractures, previous injuries.

Absent 0 Mild 1 Moderate 2 Severe 3

Q6

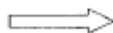
☐

Add scores for 1 - 6 and record here



Total Pain Score

Now tick the box that matches the
Total Pain Score



0 - 2 No pain	3 - 7 Mild	8 - 13 Moderate	14 + Severe
------------------	---------------	--------------------	----------------

Finally, tick the box which matches
the type of pain



Chronic	Acute	Acute on Chronic
---------	-------	---------------------

Abbey, J, De Bellis, A, Piller, N, Esterman, A, Giles, L, Parker, D and Lowrey, B.
Funded by the JH & JD Gunn Medical Research Foundation 1996 - 2002
(This document may be reproduced with this acknowledgment retained)

